

CONSORT 2010 checklist of information to include when reporting a randomised trial*

Section/Topic	Item No	Checklist item	Reported on page No
Title and abstract	1a	Identification as a randomised trial in the title	P1
	1b	Structured summary of trial design, methods, results, and conclusions (for specific guidance see CONSORT for abstracts)	P5
Introduction	2a	Scientific background and explanation of rationale	P6
	2b	Specific objectives or hypotheses	P6-7
Methods	3a	Description of trial design (such as parallel, factorial) including allocation ratio	P6
	3b	Important changes to methods after trial commencement (such as eligibility criteria), with reasons	P7-10
	4a	Eligibility criteria for participants	P7-8
	4b	Settings and locations where the data were collected	P7
Interventions	5	The interventions for each group with sufficient details to allow replication, including how and when they were actually administered	P7-10 and Study protocol (Czigany et al. BMJ Open 2017 PMID: 29018070)
Outcomes	6a	Completely defined pre-specified primary and secondary outcome measures, including how and when they were assessed	P7-8 and Study protocol (Czigany et al. BMJ Open 2017 PMID: 29018070)
Sample size	6b	Any changes to trial outcomes after the trial commenced, with reasons	Not applicable
	7a	How sample size was determined	P9 and Study protocol (Czigany et al. BMJ Open 2017 PMID: 29018070)
	7b	When applicable, explanation of any interim analyses and stopping guidelines	P7-10 and Study protocol (Czigany et al. BMJ Open 2017 PMID: 29018070)

Randomisation:			
Sequence generation	8a	Method used to generate the random allocation sequence	P7-10 and Study protocol (Czigany et al. BMJ Open 2017 PMID: 29018070)
	8b	Type of randomisation; details of any restriction (such as blocking and block size)	P7-10 and Study protocol (Czigany et al. BMJ Open 2017 PMID: 29018070)
Allocation concealment mechanism	9	Mechanism used to implement the random allocation sequence (such as sequentially numbered containers), describing any steps taken to conceal the sequence until interventions were assigned	P7-10 and Study protocol (Czigany et al. BMJ Open 2017 PMID: 29018070)
Implementation	10	Who generated the random allocation sequence, who enrolled participants, and who assigned participants to interventions	P7-10
Blinding	11a	If done, who was blinded after assignment to interventions (for example, participants, care providers, those assessing outcomes) and how	P7-10 and Study protocol (Czigany et al. BMJ Open 2017 PMID: 29018070)
	11b	If relevant, description of the similarity of interventions	P7-10 and Study protocol (Czigany et al. BMJ Open 2017 PMID: 29018070)
Statistical methods	12a	Statistical methods used to compare groups for primary and secondary outcomes	P9-10
	12b	Methods for additional analyses, such as subgroup analyses and adjusted analyses	P9-10
Results			
Participant flow (a diagram is strongly recommended)	13a	For each group, the numbers of participants who were randomly assigned, received intended treatment, and were analysed for the primary outcome	P11 and Flowchart
	13b	For each group, losses and exclusions after randomisation, together with reasons	P7
Recruitment	14a	Dates defining the periods of recruitment and follow-up	n.a.
	14b	Why the trial ended or was stopped	

Baseline data	15	A table showing baseline demographic and clinical characteristics for each group	Suppl Table 1
Numbers analysed	16	For each group, number of participants (denominator) included in each analysis and whether the analysis was by original assigned groups	Table 1, 2 and suppl Table 1
Outcomes and estimation	17a	For each primary and secondary outcome, results for each group, and the estimated effect size and its precision (such as 95% confidence interval)	Table 1, 2 and suppl Table 1
	17b	For binary outcomes, presentation of both absolute and relative effect sizes is recommended	Table 1, 2 and suppl Table 1
Ancillary analyses	18	Results of any other analyses performed, including subgroup analyses and adjusted analyses, distinguishing pre-specified from exploratory	n.a.
Harms	19	All important harms or unintended effects in each group (for specific guidance see CONSORT for harms)	Table 1
Discussion			
Limitations	20	Trial limitations, addressing sources of potential bias, imprecision, and, if relevant, multiplicity of analyses	P17
Generalisability	21	Generalisability (external validity, applicability) of the trial findings	P15-17
Interpretation	22	Interpretation consistent with results, balancing benefits and harms, and considering other relevant evidence	P15-17
Other information			
Registration	23	Registration number and name of trial registry	P5
Protocol	24	Where the full trial protocol can be accessed, if available	P5 and Study protocol (Czigany et al. BMJ Open 2017 PMID: 29018070)
Funding	25	Sources of funding and other support (such as supply of drugs), role of funders	P18

*We strongly recommend reading this statement in conjunction with the CONSORT 2010 Explanation and Elaboration for important clarifications on all the items. If relevant, we also recommend reading CONSORT extensions for cluster randomised trials, non-inferiority and equivalence trials, non-pharmacological treatments, herbal interventions, and pragmatic trials. Additional extensions are forthcoming: for those and for up to date references relevant to this checklist, see www.consort-statement.org.